Valley Regional Hospital - Department of Clinical Services

CLINICAL DOCUMENTATION — NARRATIVE SPECIALTY CONSULT NOTE V1

Patient: William Jackson | **MRN:** PAT-60016 | **DOB:** 1953-06-22 | **Gender:** Male | **Doc ID:** NARRATIVE_0016
Encounter Date: 2025-06-01 | **Discharge/Note Date:** 2025-06-03 | **Provider:** Dr. Robert Evans, MD

REASON FOR CONSULTATION

Consult requested by primary medical team regarding refractory clinical optimization and therapeutic titration. Consultant team: Cardiovascular Medicine Consult Service.

CHRONOLOGICAL CLINICAL NARRATIVE

CHRONOLOGICAL CLINICAL NARRATIVE: We were asked to evaluate patient William Jackson, who was admitted three days ago with complex multisystem decompensation. On initial presentation, initial biomarker profiling revealed hemoglobin of 9.82 g/dL (LOW). Subsequent evaluation demonstrated that serum sodium was 149.2 mmol/L (HIGH). Prior outpatient regimens had proven insufficient to achieve therapeutic stability.

HOSPITAL COURSE & TRAJECTORY

HOSPITAL COURSE & TRAJECTORY: During inpatient specialty evaluation, the consultant team adjusted pharmacological therapy. The patient was initiated on Metformin 850 mg oral twice daily. Following this intervention, trajectory assessment showed platelet count of 520.9 10*3/uL (HIGH). Clinical response has been favorable with marked improvement in objective metabolic parameters.

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS: The specialty team recommends the following maintenance regimen: continue Metoprolol 25 mg PO twice daily; and titrate Omeprazole 20 mg PO once daily in the morning. Avoid NSAIDs and nephrotoxic exposures.

CONSULTANT ASSESSMENT & SPECIALTY PLAN

The patient is stabilized and appropriate for transfer to the regular step-down floor. Re-evaluate laboratory markers in 48 hours. Thank you for this interesting consultation.